

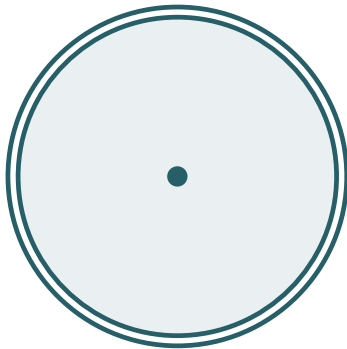
Vision that a chart can miss

CLINICAL M3-M5 | TYPE B | A focused decision guide

Match the test to the task

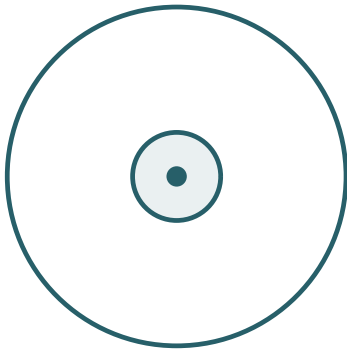
Same acuity different function

Broad field



Central acuity 6/6

Constricted field



Central acuity 6/6

Dots mark fixation; shaded areas illustrate usable field.

Original functional schematic. These field circles are conceptual, not measured patient perimetry.

Use the complaint

Difficulty	Assessment or support
Collisions despite 6/6	Fields and navigation; orientation and mobility support.
Cannot read despite stable scar	Near task, contrast and lighting; reading aids.
Colour or brightness asymmetry	Pupils, colour and optic-nerve assessment.

Interpret the whole examination

Central acuity alone cannot describe disability or exclude optic-nerve and retinal disease. A clear optical explanation must account for the pupil and field findings too.

Positive evidence without an accusation

Functional visual symptoms

- 1 Complete the ocular and neurological assessment; organic disease can coexist.
- 2 Demonstrate preserved function using appropriate positive tests, such as reliable stereopsis or internally inconsistent fields.
- 3 Explain the preserved function calmly and arrange follow-up; investigate new objective abnormalities.

What follows from the evidence

Observation	Reasonable conclusion
Better function demonstrated	The reported deficit does not match measured capacity.
Normal fundus alone	Insufficient to diagnose functional symptoms.
Inconsistency or incentive	Neither alone proves conscious deception.

Communication

Describe findings accurately. Avoid inferring malingering solely from an inconsistent chart response.